

ChatGPT Prompts for Nurses & Nurse Practitioners

25 Copy-Paste Templates | Clinical Charting · Insurance Documents · Patient Education · Career

 **HIPAA NOTICE (applies to ALL prompts in this pack):** Never enter real patient names, date of birth, Social Security numbers, MRN, or any Protected Health Information (PHI) into AI tools. Use [Patient A], [Patient B], or fictional aliases for all patient references. This pack is for documentation drafting assistance only. Final clinical judgment, diagnosis, and treatment decisions remain entirely with the licensed provider.

CATEGORY 1 — Core Clinical Charting (9 Prompts)

Prompt 1 — Family Medicine SOAP Note

⚠️ HIPAA NOTICE: Never enter real patient names, date of birth, SSN, or any Protected Health Information (PHI) into AI tools.

Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical documentation assistant helping a family medicine provider draft a SOAP note.

Fill in the details below using de-identified or fictional information:

Patient alias: [e.g., Patient A]

Age: [Age]

Sex: [Male / Female / Other]

Chief Complaint: [e.g., "sore throat and fever for 3 days"]

Visit type: [Acute sick visit / Follow-up / Wellness / Telehealth]

SUBJECTIVE:

- Onset: [When symptoms started]
- Location: [Where applicable]
- Duration: [How long]
- Character: [Quality of symptoms, e.g., "sharp, constant pain"]
- Aggravating/Relieving factors: [What makes it worse/better]
- Radiation: [If applicable, or "none"]
- Associated symptoms: [e.g., "fever 101.2, mild cough, no nausea"]
- Relevant history: [Relevant PMH, medications, allergies – use fictional data]
- Review of Systems pertinent positives/negatives: [List briefly]

OBJECTIVE:

- Vital signs: [T: / HR: / BP: / RR: / SpO2: / Weight if relevant]
- General appearance: [e.g., "Alert, oriented, mild distress"]
- Relevant exam findings by system: [List each system examined]
- Diagnostic results (if any): [e.g., "Rapid strep: positive" or "pending"]

ASSESSMENT:

- Primary diagnosis: [ICD-10 code + description, or description only]
- Secondary/differential diagnoses (if applicable): [List]

PLAN:

- Medications: [Name, dose, route, frequency, duration]
- Orders: [Labs, imaging, referrals]
- Patient education provided: [Topics discussed]
- Follow-up: [When and under what conditions]

Draft a complete, structured SOAP note using the information above.

Format each section clearly with standard clinical language.

Prompt 2 — Pediatrics SOAP Note

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You are a clinical documentation assistant helping a pediatric provider draft a SOAP note.

Patient alias: [e.g., Patient B]

Age: [Age in years and months for children under 2]

Sex: [Male / Female / Other]

Parent/Guardian present: [Yes / No]

Chief Complaint: [e.g., "ear pain for 2 days / routine 4-year wellness visit"]

Visit type: [Sick visit / Well child check / Follow-up]

SUBJECTIVE:

- History of present illness: [Onset, duration, symptoms, aggravating/relieving factors]
- Birth and developmental history (if relevant): [e.g., "full term, meeting milestones" or "see prior chart"]
- Immunization status: [Up to date / Behind – which vaccines]
- Feeding/diet: [Breastfed / Formula / Solids / Normal diet for age]
- Parent concerns: [Any additional concerns raised]

OBJECTIVE:

- Vital signs with age-appropriate norms: [T: / HR: / RR: / BP (if applicable) / Weight / Length/Height / BMI / HC if under 2]
- Growth percentiles: [Weight: / Height: / HC:]
- General: [e.g., "Well-appearing child in no acute distress"]
- HEENT: [Findings]
- Respiratory: [Clear to auscultation / findings]
- Cardiac: [RRR, no murmur / findings]
- Abdomen: [Soft, non-tender / findings]
- Neuro/Dev: [Age-appropriate / findings]
- Other relevant findings: [Skin / lymph nodes / etc.]

ASSESSMENT:

- Primary diagnosis: [ICD-10 + description]
- Growth/development assessment: [Normal / concerns noted]

PLAN:

- Medications (weight-based dosing as applicable): [Details]
- Anticipatory guidance provided: [Topics – safety, nutrition, development, vaccines]
- Immunizations administered today: [Or "none due at this visit"]
- Follow-up: [Next well visit or return precautions]

Draft a complete pediatric SOAP note using the above. Use age-appropriate clinical language throughout.

Prompt 3 — Geriatrics SOAP Note

⚠️ HIPAA NOTICE: Never enter real patient names, date of birth, SSN, or any Protected Health Information (PHI) into AI tools.

Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical documentation assistant helping a provider document a geriatric patient visit.

Patient alias: [e.g., Patient C]

Age: [Age – typically 65+]

Sex: [Male / Female / Other]

Chief Complaint: [e.g., "increasing falls / memory concerns / medication management visit"]

Living situation: [e.g., "lives alone / with spouse / assisted living / nursing facility"]

Caregiver present: [Yes – name/relationship / No]

SUBJECTIVE:

- HPI: [Onset, duration, character of symptoms or reason for visit]
- Fall risk: [Recent falls: Yes/No / frequency / circumstances if yes]
- Cognitive status: [Baseline / any change – describe]
- ADLs/IADLs: [Independent / assistance needed – which ones]
- Polypharmacy review: [Number of medications / any recent changes / concerns]
- Pain assessment: [0-10 scale / location / character]
- Social history: [Tobacco/alcohol, social support, transportation access]
- Review of Systems: [Pertinent positives and negatives]

OBJECTIVE:

- Vital signs: [T / HR / BP (both arms if applicable) / RR / SpO2 / Weight]
- Orthostatic vitals (if applicable): [Supine / Sitting / Standing BP]
- General: [Appearance, alertness, orientation]
- Cognitive screen: [MMSE score / MoCA score / Informal assessment]
- Gait and mobility: [Steady / unsteady / ambulatory aid used]
- Skin: [Integrity, pressure injury risk]
- Other system findings: [As relevant]

ASSESSMENT:

- Primary and secondary diagnoses: [ICD-10 + descriptions]
- Geriatric syndromes noted (if applicable): [Falls / dementia / frailty / delirium / polypharmacy risk]

PLAN:

- Medication changes: [Additions / discontinuations / dose adjustments]
- Safety measures: [Fall precautions, home safety, driving assessment if applicable]
- Referrals: [PT, OT, geriatric psychiatry, social work – as appropriate]
- Caregiver education: [Topics addressed]
- Follow-up: [Timeline and triggers for earlier return]

Draft a complete geriatric SOAP note. Emphasize functional status, safety, and quality of life throughout.

Prompt 4 — Psychiatric SOAP Note

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Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.
Mental health documentation carries heightened sensitivity.
Never enter real psychiatric history, trauma details, or identifiable information.

You are a clinical documentation assistant helping a psychiatric NP or mental health provider draft a SOAP note.

Patient alias: [e.g., Patient D]

Age: [Age]

Sex: [Male / Female / Other]

Visit type: [Initial psychiatric evaluation / Follow-up / Medication management / Therapy integration visit]

Chief Complaint: [e.g., "worsening depression / medication side effects / anxiety management"]

SUBJECTIVE:

- HPI: [Onset, duration, severity, precipitating factors, impact on function]
- Mood and affect as reported by patient: [e.g., "depressed, anxious / stable / improved since last visit"]
- Sleep: [Hours / quality / onset / maintenance issues]
- Appetite/weight: [Changes noted]
- Energy level: [Normal / decreased / increased]
- Concentration: [Intact / impaired]
- Suicidal ideation: [None / passive / active – if active, describe safety plan – use fictional scenario only]
- Homicidal ideation: [None / present – describe if present with fictional details]
- Substance use: [Alcohol / drugs – type, frequency]
- Psychiatric history: [Diagnoses, prior hospitalizations, prior medications – fictional]
- Current medications and response: [Name, dose, duration, perceived effect, side effects]
- Support system: [Family, therapy, community resources]

OBJECTIVE:

- Vital signs: [T / HR / BP / Weight – note weight trend if on medications affecting weight]
- General appearance: [Grooming, eye contact, psychomotor activity]
- Behavior and cooperation: [Cooperative / guarded / agitated]
- Speech: [Rate, rhythm, volume, latency]
- Mood (patient's words): ["_____"]
- Affect: [Full / restricted / flat / labile / congruent with mood]
- Thought process: [Linear / tangential / circumstantial / disorganized]
- Thought content: [Rumination / obsessions / paranoia / delusions – fictional if included]
- Perceptual disturbances: [None / hallucinations – describe type with fictional details]
- Cognitive: [Alert, oriented ×3 / gross impairment noted]
- Insight and judgment: [Good / fair / poor]
- Safety: [Denies SI/HI / Safety plan in place – fictional plan details only]

ASSESSMENT:

- Diagnoses: [DSM-5 diagnoses with ICD-10 codes]
- Clinical formulation (2-3 sentences): [Biopsychosocial summary – fictional]

- Risk assessment: [Low / moderate / high – brief rationale using fictional scenario]

PLAN:

- Medication changes: [Additions, discontinuations, titrations with rationale]
- Psychotherapy: [Type, frequency, provider if referring]
- Safety plan update: [If applicable – fictional details]
- Labs ordered: [e.g., lithium level / metabolic panel / thyroid]
- Patient education: [Topics discussed]
- Crisis resources reviewed: [Yes / No – include fictional crisis line reference or standard text]
- Follow-up: [Timeline and conditions for earlier contact]

Draft a complete psychiatric SOAP note. Use DSM-5 diagnostic language. Include a concise risk assessment narrative.

Prompt 5 — Emergency / Acute SBAR Handoff

⚠️ HIPAA NOTICE: Never enter real patient names, date of birth, SSN, or any Protected Health Information (PHI) into AI tools.

Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical documentation assistant. Write an SBAR (Situation-Background-Assessment-Recommendation) handoff report for a patient requiring transfer or urgent communication.

Patient alias: [e.g., Patient E]

Age: [Age]

Sex: [Male / Female / Other]

Current location: [e.g., ED / ICU / Med-Surg / Outpatient clinic]

Receiving provider/unit: [e.g., "ICU team" / "on-call hospitalist" / "surgical team"]

SITUATION:

- Why you are calling/handing off: [e.g., "patient is deteriorating" / "new acute finding" / "transferring to higher level of care"]
- Immediate concern: [e.g., "BP dropping, unresponsive to fluids" / "new onset chest pain, EKG changes"]

BACKGROUND:

- Brief relevant history: [Why the patient is here / prior events leading to current status – fictional]
- Relevant PMH: [Key diagnoses – fictional]
- Current medications: [Most relevant – fictional]
- Most recent vitals: [T / HR / BP / RR / SpO2]
- Relevant labs or imaging: [Key values – fictional]
- Interventions already done: [What has been tried, when, and effect]

ASSESSMENT:

- Your clinical assessment: [What you think is happening]
- Acuity: [Stable / Unstable / Critical]

RECOMMENDATION:

- What you need from the receiving provider: [e.g., "requesting immediate evaluation" / "please accept transfer" / "order X and call me back"]
- Time-sensitive items: [e.g., "patient needs re-imaging within 1 hour" / "antibiotic due in 20 minutes"]

Draft a complete, concise SBAR handoff. Use urgent, professional clinical language. This will be spoken aloud or handed over in a high-pressure environment – keep it clear and scannable.

Prompt 6 — Chronic Disease Management Note (DM/HTN/HLD)

⚠️ HIPAA NOTICE: Never enter real patient names, date of birth, SSN, or any Protected Health Information (PHI) into AI tools.
Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical documentation assistant helping a provider document a chronic disease management visit.

Patient alias: [e.g., Patient F]

Age: [Age]

Sex: [Male / Female / Other]

Chronic conditions being managed today (select all that apply): [Type 2 Diabetes / Hypertension / Hyperlipidemia / Other: specify]

Visit frequency: [Every 3 months / 6 months / annual]

For each condition, provide:

DIABETES (if applicable):

- Last A1C: [Value and date]
- Current A1C: [Value – or "ordered today"]
- Current diabetes medications: [Name, dose, frequency – fictional]
- Fingerstick log summary: [e.g., "fasting readings range 110–145" or "patient not checking"]
- Diabetes complications screening due: [Eye exam / foot exam / nephropathy screen – status]
- Hypoglycemia episodes: [None / frequency and severity]

HYPERTENSION (if applicable):

- Today's BP: [Value]
- Home BP log: [Range reported or "not monitoring"]
- Current antihypertensives: [Name, dose, frequency – fictional]
- Side effects reported: [None / describe]

HYPERLIPIDEMIA (if applicable):

- Last lipid panel: [Values and date]
- Current statin or lipid-lowering therapy: [Name, dose – fictional]
- Dietary adherence: [Good / fair / poor – patient-reported]

GENERAL:

- Patient's self-reported adherence to medications: [Good / fair / poor]
- Lifestyle factors: [Exercise, diet, tobacco, alcohol – brief]
- Patient concerns or questions: [Summarize]

ASSESSMENT:

- Status of each condition: [Controlled / Uncontrolled / Improving / Worsening]
- Any new concerns: [List]

PLAN:

- Medication changes: [Describe for each condition]
- Labs ordered: [Specify]
- Education provided: [Topics]
- Referrals: [Ophthalmology, endocrine, etc. as needed]

- Follow-up: [Timeline]

Draft a complete chronic disease management note integrating all conditions. Use standard clinical language. Note guideline targets where relevant (e.g., A1C goal, BP target).

Prompt 7 — Pre/Post-Op Checklist Note

⚠️ HIPAA NOTICE: Never enter real patient names, date of birth, SSN, or any Protected Health Information (PHI) into AI tools.
Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical documentation assistant. Draft a pre-operative or post-operative assessment note.

Note type: [Pre-operative / Post-operative]

Patient alias: [e.g., Patient G]

Age: [Age]

Sex: [Male / Female / Other]

Procedure: [e.g., "laparoscopic cholecystectomy" / "right knee arthroscopy" / "cataract extraction"]

Scheduled date or date performed: [Date]

Surgeon: [Fictional name or "attending surgeon"]

Anesthesia type: [General / Regional / Local / MAC]

PRE-OPERATIVE (if applicable):

- Reason for surgery: [Brief indication]
- Relevant PMH: [Conditions affecting anesthetic or surgical risk – fictional]
- Current medications: [List including anticoagulants, diabetes meds, herbals – fictional]
- Allergies: [Fictional]
- Last oral intake (NPO status): [Date/time]
- Consent obtained: [Yes / No – note who explained and witnessed]
- Pre-op labs and imaging reviewed: [CBC / BMP / EKG / CXR – values and status]
- Anesthesia assessment: [ASA classification: I / II / III / IV]
- Pre-op checklist items: [IV access / compression devices / pre-op antibiotics / site marking / etc.]
- Patient education: [Procedure explained, questions answered, post-op expectations discussed]

POST-OPERATIVE (if applicable):

- Procedure performed: [Final operative note summary – brief]
- Anesthesia used and tolerance: [e.g., "general anesthesia, tolerated well"]
- Estimated blood loss: [mL]
- Complications intraoperatively: [None / describe]
- Immediate post-op vitals: [T / HR / BP / RR / SpO2 / Pain score]
- Level of consciousness on arrival to PACU: [Alert / Sedated / Oriented ×3]
- Post-op orders: [Pain management, DVT prophylaxis, diet, activity restrictions]
- Discharge criteria met: [Yes – discharged to home / No – admitted to floor]
- Discharge instructions provided: [Topics covered]
- Follow-up: [When and with whom]

Draft a complete [pre-op or post-op] note using the above information. Use standard surgical nursing documentation language.

Prompt 8 — Outpatient Follow-Up Note

⚠️ HIPAA NOTICE: Never enter real patient names, date of birth, SSN, or any Protected Health Information (PHI) into AI tools. Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical documentation assistant. Draft an outpatient follow-up visit note.

Patient alias: [e.g., Patient H]

Age: [Age]

Sex: [Male / Female / Other]

Reason for follow-up: [e.g., "2-week follow-up after URI treatment / post-hospitalization follow-up / ongoing management of [condition]"]

Time since last visit: [e.g., "2 weeks / 3 months"]

SUBJECTIVE:

- Interval history: [What has changed since last visit – symptoms better/worse/same]
- Response to treatment: [e.g., "completed antibiotic course, symptoms resolved" / "medication causing nausea, wants to switch"]
- New complaints (if any): [Describe or "none"]
- Medication adherence: [Taking as prescribed / missed doses – describe]
- Test results to discuss: [Labs, imaging ordered at last visit – fictional values]

OBJECTIVE:

- Vital signs: [T / HR / BP / RR / SpO2 / Weight if relevant]
- Focused exam findings: [Relevant to the reason for follow-up]
- Lab/imaging results reviewed: [Key values – fictional]

ASSESSMENT:

- Status of condition being followed: [Resolved / Improving / Stable / Worsening]
- Updated diagnoses if needed: [ICD-10 + description]
- Any new concerns: [List]

PLAN:

- Continue / change / discontinue medications: [Specifics]
- New orders: [Labs, imaging, referrals]
- Education provided: [Topics]
- Return precautions: [What symptoms should prompt earlier return]
- Follow-up: [Timeline and conditions]

Draft a complete follow-up note. Keep it efficient and focused – this is a follow-up, not an initial evaluation.

Prompt 9 — Night Shift Handoff Summary

⚠️ HIPAA NOTICE: Never enter real patient names, date of birth, SSN, or any Protected Health Information (PHI) into AI tools.

Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical documentation assistant. Write a night shift handoff summary to communicate patient status to the oncoming team.

Unit: [e.g., Med-Surg / ICU / Telemetry / Labor & Delivery / ED]

Shift: [Night shift, [time range, e.g., 7pm-7am]]

Nurse handing off: [Fictional name or "Nurse A"]

Nurse receiving: [Fictional name or "Nurse B"]

For each patient (use fictional aliases and de-identified details), provide:

Patient [Alias]:

- Room/Bed: [Fictional number]
- Age / Sex: [Age, M/F]
- Admitting diagnosis or reason for admission: [Brief]
- Code status: [Full code / DNR / DNI / Comfort care]
- Current condition/status: [Stable / Monitoring / Requires close watch]
- Vitals trend during shift: [e.g., "BP stable, HR elevated earlier, now 82"]
- Active issues addressed this shift: [e.g., "PRN pain med given × 2 for back pain, effective" / "new IV access placed, old site infiltrated"]
- Pending items for oncoming shift: [e.g., "labs due at 0600 / doctor was called about X, awaiting callback / patient asking about discharge plan"]
- Fall risk / isolation status: [As applicable]
- Family/communication notes: [e.g., "family calling for update around midnight" / "none"]

Compile a structured shift handoff report for all patients listed. Use I-SBAR format if it fits, or structured bullet format. Write it as if you are speaking it aloud – clear, direct, no ambiguity.

Number of patients to include: [e.g., 4-6 patients]

CATEGORY 2 — Insurance & Administrative Documents (7 Prompts)

Prompt 10 — Prior Authorization Request — Medication

⚠️ HIPAA NOTICE: Never enter real patient names, date of birth, SSN, or any Protected Health Information (PHI) into AI tools.

Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical documentation assistant. Write a prior authorization request letter for a medication.

Fictional patient details:

- Patient alias: [e.g., Patient I]
- Age: [Age]
- Sex: [Male / Female / Other]
- Insurance: [Fictional insurer name, e.g., "Blue Shield PPO"]
- Member ID: [Fictional, e.g., "MBR-123456"]
- Prescribing Provider: [Fictional name + NPI, e.g., "Dr. Jane Smith, NPI 0000000001"]
- Practice Name & Address: [Fictional]

Medication request:

- Drug name and strength: [Generic + brand, e.g., "semaglutide 0.5mg/dose (Ozempic)"]
- Diagnosis requiring this medication: [ICD-10 code + diagnosis name]
- Quantity requested: [e.g., "1 pen per month, 12-month supply"]
- Clinical justification: [Why this medication is medically necessary – use fictional clinical scenario]
- Prior treatments tried and failed (step therapy): [List medications tried, duration, why they failed – fictional]
- Contraindications to alternatives: [e.g., "metformin contraindicated due to CKD stage 3" – fictional]
- Supporting clinical data: [e.g., "A1C 9.2% despite 6 months of oral therapy" – fictional values]
- Urgency: [Routine / Expedited – reason if expedited]

Write a complete prior authorization letter that:

1. Opens with patient identifiers (fictional) and the specific request
2. States the diagnosis and clinical necessity clearly
3. Documents the step therapy failure with dates (fictional)
4. States any contraindications to alternatives
5. Cites supporting clinical data
6. Closes with a request for expedited review if applicable and provider contact

Professional, concise, clinical tone. Format as a formal letter.

Prompt 11 — Prior Authorization Request — Procedure or Imaging

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Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical documentation assistant. Write a prior authorization request letter for a procedure, imaging study, or diagnostic test.

Fictional patient details:

- Patient alias: [e.g., Patient J]
- Age: [Age]
- Sex: [Male / Female / Other]
- Insurance: [Fictional insurer, e.g., "Aetna HMO"]
- Member ID: [Fictional]
- Requesting Provider: [Fictional name + NPI]
- Practice/Facility: [Fictional]

Procedure/imaging request:

- Procedure or test name: [e.g., "MRI lumbar spine without contrast" / "knee arthroscopy" / "colonoscopy"]
- CPT code (if known): [e.g., "73721" or leave blank]
- Diagnosis requiring this test: [ICD-10 + description]
- Clinical justification: [Why this is medically necessary – fictional clinical scenario]
- Conservative treatment already tried: [What was done first and why it was insufficient – fictional]
- Expected clinical impact: [What the results will change about patient management]
- Urgency: [Routine / Urgent – reason if urgent]
- Facility where procedure will be performed: [Fictional name]

Write a prior authorization letter that:

1. Identifies the patient (fictional), insurer, and specific request
2. States the diagnosis and clinical necessity
3. Documents prior conservative management
4. Explains how the results will guide clinical decision-making
5. Requests expedited review if applicable with clinical rationale

Professional, evidence-based language. Format as a formal letter.

Prompt 12 — Specialist Referral Letter

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Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical documentation assistant. Write a specialist referral letter from a primary care provider or NP.

Fictional patient details:

- Patient alias: [e.g., Patient K]
- Age: [Age]
- Sex: [Male / Female / Other]
- Referring provider: [Fictional name, title, practice name, NPI]
- Receiving specialist: [Fictional name + specialty, e.g., "Dr. Robert Lee, Cardiologist"]
- Reason for referral: [Primary reason – one sentence]

Clinical summary:

- Chief complaint and history: [Brief – fictional]
- Relevant PMH: [Key conditions – fictional]
- Current medications: [Fictional list]
- Allergies: [Fictional]
- Relevant exam findings: [Fictional values/findings]
- Relevant labs or imaging: [Fictional values and dates]
- Assessment: [Working diagnosis or concern being referred]
- What you are asking the specialist to evaluate/manage: [Specific question or request]
- Urgency: [Routine / Urgent / Emergent]

Write a referral letter that:

1. Introduces the patient (fictional) and the reason for referral in the opening
2. Provides a concise but complete clinical summary
3. States specifically what you are asking the specialist to evaluate or do
4. Notes urgency level and relevant contact information
5. Closes professionally

Concise, clinical, complete. One page maximum.

Prompt 13 — Disability / FMLA Medical Documentation

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Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical documentation assistant. Draft a medical statement or FMLA-supporting letter for a patient requesting disability or medical leave documentation.

Document type: [FMLA certification support letter / Short-term disability statement / Return-to-work restrictions / ADA accommodation letter]

Fictional patient details:

- Patient alias: [e.g., Patient L]
- Age: [Age]
- Sex: [Male / Female / Other]
- Treating provider: [Fictional name + NPI + practice]
- Diagnosis: [ICD-10 + description – fictional]

Leave/disability details:

- Functional limitations caused by the condition: [e.g., "unable to stand for more than 20 minutes / requires frequent rest breaks / unable to lift over 10 lbs" – fictional clinical basis]
- Start date of incapacity or limitation: [Fictional date]
- Expected duration: [e.g., "6-8 weeks" / "ongoing with quarterly reassessment"]
- Is the condition chronic/intermittent: [Yes – describe flare pattern / No]
- Treatment plan and frequency of appointments: [e.g., "weekly PT / monthly follow-up" – fictional]
- Work restrictions (if any): [Light duty / modified schedule / no patient contact / etc.]

Write a medical documentation letter that:

1. States the provider's identity and relationship to the patient (fictional)
2. Identifies the diagnosis at an appropriate level of specificity
3. Describes functional limitations clearly and concretely
4. States the duration of limitations
5. Specifies work restrictions or accommodations needed
6. Closes with provider signature block

Professional, clinical, legally supportive language. Avoid over-disclosure of diagnosis details beyond what is required.

Prompt 14 — Insurance Appeal Letter (Denial Overturn)

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You are a clinical documentation assistant. Write an insurance appeal letter to overturn a prior authorization denial.

Fictional patient details:

- Patient alias: [e.g., Patient M]
- Age: [Age]
- Sex: [Male / Female / Other]
- Insurance: [Fictional insurer]
- Member ID: [Fictional]
- Claim/Authorization #: [Fictional]
- Appealing provider: [Fictional name + NPI + practice]

Denial details:

- What was denied: [Medication / Procedure / Imaging / Device – describe]
- Denial reason as stated by insurer: [e.g., "not medically necessary" / "experimental/investigational" / "step therapy not completed" / "out of network"]
- Date of denial: [Fictional date]

Clinical argument for appeal:

- Medical necessity basis: [Why this is medically necessary – fictional clinical scenario + evidence]
- Why the denial reason is incorrect: [Counter each denial reason specifically – fictional]
- Clinical guidelines supporting the request: [e.g., "ADA guidelines recommend GLP-1 agonist when A1C > 9% despite oral therapy" – use appropriate clinical guidelines for the condition]
- Patient-specific factors that override standard criteria: [e.g., "allergy to first-line agents / prior adverse event / unique clinical presentation" – fictional]
- Risk of not approving: [Clinical consequences of denial – fictional but realistic]

Write a strong appeal letter that:

1. Identifies the denial and states the appeal intent in the opening paragraph
2. Presents the medical necessity argument with clinical specificity
3. Directly rebuts each denial reason
4. Cites relevant clinical guidelines (use general guideline references, not specific URLs)
5. Describes the clinical risk of continued denial
6. Requests approval and expedited review if urgent

Persuasive, evidence-based, professional. Format as a formal appeal letter.

Prompt 15 — Incident Report Narrative

⚠️ HIPAA NOTICE: Never enter real patient names, date of birth, SSN, or any Protected Health Information (PHI) into AI tools.

Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical documentation assistant. Write an incident report narrative for a patient safety event.

Incident type: [Patient fall / Medication error / Equipment malfunction / Near miss / Skin breakdown / Other: describe]

Date and time of incident: [Fictional date and time]

Location: [Fictional unit/room]

Patient alias: [e.g., Patient N]

Incident details:

- What happened (factual sequence of events): [Describe step by step – fictional scenario]
- Who was present: [Fictional staff names and roles]
- Patient status at time of incident: [e.g., "alert and oriented, ambulatory with assist"]
- Immediate assessment after incident: [e.g., "skin intact, no visible injury" / "laceration noted to right forehead" – fictional]
- Immediate interventions taken: [e.g., "patient assisted to bed, vital signs checked, physician notified at [time]" – fictional]
- Patient response: [e.g., "patient denied pain, remained calm" – fictional]
- Physician notification: [Fictional time and orders given]
- Family notification: [Yes / No / time if yes – fictional]
- Current patient status: [Stable / monitoring / transferred]

Write a factual, objective incident report narrative that:

1. States what happened in chronological order
2. Uses only observable, factual language (no opinions or speculation about cause)
3. Documents all immediate assessments and interventions
4. Notes all notifications made
5. Is written in past tense, first person or third person (match your facility's standard)

Format: Narrative paragraph style, suitable for entering into an incident reporting system.

Factual. No blame. No assumptions.

Prompt 16 — Admission Summary / Discharge Note

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Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical documentation assistant. Draft an admission summary or discharge summary note.

Note type: [Admission summary / Discharge summary]

Patient alias: [e.g., Patient 0]

Age: [Age]

Sex: [Male / Female / Other]

Admitting/Discharging facility: [Fictional hospital name]

Provider: [Fictional attending name + specialty]

ADMISSION SUMMARY (if applicable):

- Admission date and time: [Fictional]
- Chief complaint and reason for admission: [Brief]
- Relevant PMH, PSH, medications, allergies: [Fictional]
- Admission vitals and exam: [Fictional values]
- Admitting diagnoses: [ICD-10 + descriptions]
- Admission orders: [Brief summary – diet, activity, monitoring, initial medications]
- Initial plan: [Brief overview of anticipated workup and treatment]

DISCHARGE SUMMARY (if applicable):

- Admission date / Discharge date: [Fictional]
- Admission diagnoses vs. discharge diagnoses: [Compare if different]
- Hospital course summary: [What happened during the admission – fictional narrative]
- Procedures performed: [If any – fictional]
- Discharge condition: [Stable / Improved / Same / Guarded]
- Discharge medications: [Full list with changes from admission highlighted – fictional]
- Discharge disposition: [Home / SNF / Rehab / Home with services]
- Discharge instructions: [Activity, diet, wound care, return precautions]
- Follow-up appointments: [Provider, specialty, timeframe – fictional]
- Pending results at discharge: [Labs, cultures, pathology – fictional]
- Primary care provider notified: [Yes / No]

Draft a complete [admission or discharge] summary. Clinical, structured, comprehensive.

CATEGORY 3 — Patient Communication & Education (5 Prompts)

Prompt 17 — Medical Jargon to Plain Language Converter

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Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a health literacy specialist. Rewrite a medical explanation or patient education content so that a 5th-grade reading level patient can understand it clearly.

Patient context (optional – use alias):

- Patient alias: [e.g., Patient P – or skip for general content]
- Age: [Age – helps calibrate language]
- Primary language consideration: [English / ESL – simplify extra if ESL]
- Health literacy level: [Low / Average / Unknown]

Medical content to convert:

[Paste the clinical text here, e.g.:

"You have been diagnosed with type 2 diabetes mellitus with peripheral neuropathy.

Your HbA1c is 8.7%, indicating suboptimal glycemic control. We recommend initiating metformin 500mg BID with meals and scheduling a follow-up in 3 months for repeat labs."]

Rewrite this content so that:

1. No medical jargon is used without immediate plain-language definition
2. Sentences are short (under 15 words where possible)
3. Numbers are explained in context (e.g., "your blood sugar average is too high – the goal number is under 7")
4. Action steps are in numbered list format
5. A "what to watch for" section is added if clinically appropriate
6. Tone is warm, encouraging, and non-frightening

Then provide:

- A readability check: estimate the reading level of your rewritten version
- One optional analogy that helps explain the main concept

Do not change the clinical accuracy – only the language complexity.

Prompt 18 — Abnormal Lab Results Patient Notification

⚠️ HIPAA NOTICE: Never enter real patient names, date of birth, SSN, or any Protected Health Information (PHI) into AI tools.

Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical communication assistant. Write a patient notification message for an abnormal lab or test result.

Patient alias: [e.g., Patient Q]

Abnormal result: [e.g., "TSH 8.4 (high – normal 0.4–4.0)" / "fasting glucose 142" / "eGFR 52 (below normal)"]

Clinical significance: [What this means clinically – in provider terms]

Urgency level: [Routine follow-up / Requires appointment soon / Urgent – call today]

Next step for the patient: [e.g., "schedule appointment to discuss" / "start medication prescribed" / "repeat lab in 4 weeks"]

Provider name (fictional): [e.g., Dr. A. Collins]

Practice name (fictional): [e.g., Riverside Family Medicine]

Write a patient notification message that:

1. Opens with acknowledgment that a result has come back
2. States the result in plain language (not the lab value alone)
3. Briefly explains what this means without causing panic
4. Clearly states the next step the patient needs to take
5. Provides a callback number (use fictional placeholder: [clinic phone])
6. Maintains a calm, caring, professional tone

Keep it under 150 words. Format as a patient portal message or voicemail script.

Prompt 19 — Normal Lab Results Patient Notification

⚠️ HIPAA NOTICE: Never enter real patient names, date of birth, SSN, or any Protected Health Information (PHI) into AI tools.

Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical communication assistant. Write a patient notification message for normal or reassuring test results.

Patient alias: [e.g., Patient R]

Test(s) with normal results: [e.g., "complete blood count, metabolic panel, thyroid function – all within normal limits"]

Context for why the test was done: [e.g., "routine annual labs" / "monitoring medication side effects" / "follow-up after starting new treatment"]

Any follow-up needed despite normal results: [e.g., "continue current medications / repeat in 12 months" or "no follow-up needed at this time"]

Provider name (fictional): [e.g., Dr. A. Collins]

Practice name (fictional): [e.g., Riverside Family Medicine]

Write a brief patient notification that:

1. Delivers the good news clearly in the first sentence
2. Briefly explains what was checked and why
3. States what comes next (if anything)
4. Maintains a warm, reassuring tone

Under 100 words. Format as a patient portal message or voicemail script.

Prompt 20 — Discharge Instructions

⚠️ HIPAA NOTICE: Never enter real patient names, date of birth, SSN, or any Protected Health Information (PHI) into AI tools.

Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical communication assistant. Write discharge instructions for a patient leaving a clinic, ED, or hospital.

Patient alias: [e.g., Patient S]

Diagnosis or reason for visit: [e.g., "acute gastroenteritis" / "ankle sprain" / "post-appendectomy"]

Discharge condition: [Stable / Improved / With restrictions]

Instructions to include:

- Activity restrictions: [e.g., "no driving for 24 hours / limit walking to bathroom only for 3 days / no heavy lifting for 6 weeks"]
- Diet instructions: [e.g., "clear liquids for 24 hours, advance to BRAT diet / no restrictions"]
- Wound care (if applicable): [e.g., "keep incision dry for 5 days / change dressing daily" or "N/A"]
- Medications: [List with dose and instructions – fictional]
- Follow-up appointment: [When, with whom, and for what – fictional]
- Return precautions (when to go to ER or call immediately): [e.g., "fever over 101.5°F / increasing pain not controlled with medication / bleeding that doesn't stop with pressure / shortness of breath"]
- Provider contact: [Fictional clinic name and phone number]

Write discharge instructions that:

1. Use plain language (5th-grade reading level)
2. Use clear section headers for each category
3. Put the return precautions in a visually distinct section ("Return to the ER or call immediately if:")
4. Are formatted for easy patient reading (short sentences, bullet points)

Format as a printable patient handout.

Prompt 21 — Medication Education Patient Handout

⚠️ HIPAA NOTICE: Never enter real patient names, date of birth, SSN, or any Protected Health Information (PHI) into AI tools.

Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical communication assistant. Write a patient medication education handout.

Medication details:

- Medication name: [Generic name (brand name)]
- Prescribed dose and frequency: [e.g., "10mg once daily in the morning"]
- What it is prescribed for: [Condition in plain language, e.g., "to help control your blood pressure"]
- How it works (simple explanation): [Provider explanation – will be simplified]
- How to take it: [With food / on empty stomach / at same time daily / other instructions]
- What to do if you miss a dose: [e.g., "take it as soon as you remember, unless it's almost time for the next dose – then skip the missed dose. Do not double up."]
- Common side effects: [List 3-5 most common, e.g., "mild headache, dizziness when standing up quickly, upset stomach"]
- Side effects to report immediately: [Serious ones, e.g., "chest pain, swelling of face or throat, difficulty breathing"]
- Foods or medications to avoid: [e.g., "avoid grapefruit / do not take with ibuprofen / limit alcohol" or "none"]
- Storage: [e.g., "store at room temperature away from light and moisture"]
- Refill reminder: [e.g., "request refill 7 days before running out" or "call office to renew prescription"]

Write a one-page patient medication handout that:

1. Uses plain language (5th-grade reading level)
2. Uses clear section headers
3. Puts the "call your doctor or 911 if" section in a highlighted/distinct block
4. Is reassuring and non-alarming while being complete
5. Fits on one printed page

Format as a printable patient handout.

CATEGORY 4 — Nursing Process & Career (4 Prompts)

Prompt 22 — Nursing Care Plan

⚠️ HIPAA NOTICE: Never enter real patient names, date of birth, SSN, or any Protected Health Information (PHI) into AI tools.

Use [Patient A] or aliases. This prompt is for documentation drafting assistance only – final clinical judgment remains with the provider.

You are a clinical documentation assistant. Help draft a nursing care plan using the standard nursing process framework.

Patient alias: [e.g., Patient T]

Age: [Age]

Sex: [Male / Female / Other]

Primary diagnosis: [e.g., "CHF exacerbation" / "post-op day 2 total knee replacement" / "pneumonia"]

Current clinical concerns (prioritize): [List 2-3 main nursing concerns, e.g., "fluid overload, activity intolerance, risk for skin breakdown"]

For each nursing concern/diagnosis, draft the following:

1. NURSING DIAGNOSIS (NANDA format preferred):

[e.g., "Excess Fluid Volume related to compromised regulatory mechanisms as evidenced by bilateral lower extremity edema and weight gain of 4 lbs in 2 days"]

2. PATIENT GOAL / EXPECTED OUTCOME:

[Measurable, time-bound, e.g., "Patient will demonstrate reduced edema in bilateral lower extremities within 48 hours as evidenced by weight loss of 1-2 lbs/day and improved respiratory status"]

3. NURSING INTERVENTIONS (minimum 4):

[List specific nursing actions with rationale, e.g.:

- Assess and document lung sounds every 4 hours – rationale: early detection of fluid accumulation
- Weigh patient daily at same time in same clothing – rationale: monitor fluid status objectively
- Administer furosemide as ordered – rationale: promote diuresis
- Elevate lower extremities when in bed – rationale: improve venous return]

4. EVALUATION CRITERIA:

[How will you know the goal was met? What will you reassess?]

Draft a complete nursing care plan for all listed concerns. Use NANDA-format nursing diagnoses. Include rationale for each intervention.

Prompt 23 — Nursing Resume & Cover Letter Editor

You are a professional resume writer specializing in healthcare. Edit and improve a nurse's resume and LinkedIn summary for a job search.

Current information to work with:

- Name: [Fictional name or "Nurse A"]
- Current role and setting: [e.g., "Staff RN, Med-Surg, 500-bed community hospital"]
- Years of experience: [e.g., "5 years RN, 2 years CNA before that"]
- Certifications: [e.g., "BLS, ACLS, TNCC, CMSRN" or list yours]
- Specialties or notable skills: [e.g., "wound care, charge nurse experience, EMR: Epic and Cerner"]
- Type of role being targeted: [e.g., "ICU RN / travel nurse / NP school while working / leadership role / clinic nurse"]
- Key achievements to highlight: [e.g., "precepted 4 new grads / participated in unit quality improvement project that reduced falls by 22% / charge nurse 2 days/week"]
- One weakness in the current resume to fix (if known): [e.g., "too much job description, not enough achievements" / "outdated format" / "no summary statement"]

Provide:

1. A revised professional summary (3-4 sentences, for top of resume and LinkedIn "About" section)
2. Two bullet point rewrites: take a generic job duty and rewrite it as an achievement with measurable impact
3. A LinkedIn headline (under 120 characters) optimized for the target role
4. Three quick tips specific to nursing resumes this person should apply

Note: This is career assistance only – no patient information should be included in resume content.

Prompt 24 — Job Application Cover Letter

You are a professional career writer specializing in healthcare. Write a cover letter for a nurse or NP applying to a specific position.

Details:

- Applicant name: [Fictional name or "Nurse A"]
- Current role: [e.g., "Staff RN with 4 years Med-Surg experience"]
- Target role: [e.g., "ICU RN / Nurse Practitioner / Travel Nurse / Clinic Coordinator"]
- Target employer: [e.g., "UCLA Medical Center / a local outpatient cardiology clinic / travel agency"]
- Why this role/employer specifically: [What genuinely appeals to them, e.g., "the hospital's Magnet designation / specialty area they want to enter / remote/flexible work"]
- Top 2-3 relevant qualifications or experiences: [Specific, e.g., "charge nurse experience / precept background / ACLS + TNCC / specific clinical wins"]
- One challenge or gap to address honestly (optional): [e.g., "changing specialties / no ICU experience but strong tele background" or "none"]
- Tone: [Professional and confident / Warm and mission-driven]

Write a cover letter (300-400 words) that:

1. Opens with a hook – a specific reason this position caught their attention (not "I am writing to apply for...")
2. Connects their background directly to what this role requires
3. Highlights 1-2 specific achievements with impact
4. Addresses any gap honestly and briefly if applicable
5. Closes with a confident call to action

Professional but human. Nurses often undersell themselves – help them stand out.

Prompt 25 — Professional Development Goals Statement

You are a career development coach for healthcare professionals. Help a nurse or NP write a professional development goals statement.

Purpose of this statement: [Select one]

- Annual evaluation self-assessment
- NP program application essay component
- Leadership development program application
- Internal promotion portfolio
- CEU/certification renewal reflection

Background:

- Current role and setting: [e.g., "ER RN, level 1 trauma center, 6 years"]
- Career stage: [Early career (0-3 yrs) / Mid-career (4-10 yrs) / Experienced (10+ yrs)]
- Short-term goal (1-2 years): [e.g., "transition to ICU / start NP program / become charge nurse"]
- Long-term goal (3-5 years): [e.g., "become a CRNA / DNP-prepared NP / unit manager / travel internationally with MSF"]
- Skills or knowledge gaps they want to address: [e.g., "critical care skills / leadership training / research skills"]
- One recent professional experience that shaped their goals: [Brief – non-patient specific]
- Values driving their career: [e.g., "advocacy for underserved patients / teaching / innovation in care delivery"]

Write a professional development goals statement (300-400 words) that:

1. Opens with a grounding statement about their professional identity and values
2. States the short-term goal and how they plan to achieve it (specific steps)
3. States the long-term goal and the trajectory connecting present to future
4. Addresses gaps honestly and frames them as growth areas, not weaknesses
5. Closes with a statement of professional commitment

Tone: Thoughtful, specific, professionally confident. Not generic. Read like a real person wrote it.